EasySLR Decision Brief

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We should spend the three discovery calls on Pharmacoevidence, Source Health Economics and ICON Health Economics, with Pharmacoevidence as the one-call pick.

1. Segment

We chose HEOR and SLR service providers: consultancies that run systematic literature reviews (SLRs) for pharma and health technology work. Their problem is slow screening, which an AI platform could cut (assumption, to confirm with EasySLR). Users are SLR leads and screening researchers; approvers are practice heads, plus procurement or IT security. We rejected pharma in-house HEOR teams: more volume, but slower procurement.

2. Top three

1. Pharmacoevidence (Mohali, India). SLRs are core work. Its own AI tool was used in a NICE UK submission (November 2025 poster), so AI is already in their workflow. Risk: they may not need an outside platform. First question: "Do you use any third-party SLR platform for work your in-house tool does not cover?"

2. Source Health Economics (Oxford, UK). About 33 staff, SLR and HTA services. Why now: it was sold to SCIRIS, so tool choices may be changing. Risk: decisions may sit with SCIRIS. First question: "After the SCIRIS deal, who chooses your review software?"

3. ICON Health Economics (UK and US). The weakest of the three: two Verified facts, one from an expired job post, so the tool says Research more. Risk: a large CRO's procurement would likely slow a pilot. First question: "Is your SLR reviewer team still hiring, and how many reviews does it deliver a year?"

3. Deferred and excluded

- **Costello Medical: Defer.** Built its own screening application.
- **Maven: Research more.** Entity and headcount unconfirmed.
- **Evalueserve: Exclude.** Sells a competing platform, LitVerse.

4. Changed evidence

Source was sold to SCIRIS, a group with over 300 staff. We kept it in the top three but flagged that decisions may sit at group level, so the call should test that first.

5. One-call pick

Pharmacoevidence. The rule engine suggests Defer because of its in-house tool. We overrode it to a call, to test whether they would use an external platform or act as a channel.

This override is the weakest judgement in the brief. Switch to Source if they rule out third-party tools.

6. Outreach and discovery

Outreach draft (not to be sent):

> Hi team,
>
> I'm researching how SLR teams handle screening in health economics work. Would you be open to a 20-minute call next week about your review tooling?
>
> Harsh Singh

Three discovery questions:

1. Which review tools do you use, and where does your in-house tool fall short?
2. Would you evaluate a third-party platform alongside your own tool, or only as a partner or channel?
3. Who decides on review software, and do group or client contracts limit the choice?

7. One-day pilot

- **Morning:** set up one live SLR and import its reference list.
- **Afternoon:** screen about 100 records alongside the team's current method.

Success (to agree with the team): at least 20% less screening time per record, and at least 90% agreement with the team's decisions.